

Mary's Hands Network

Postpartum Doula Service Guide

Internal Instructions for Postpartum Doulas

About This Guide

This guide contains internal instructions for Mary's Hands Network postpartum doulas. It explains the two pathways clients use to enter postpartum services, the full visit sequence from the virtual introductory visit through three months postpartum, and your scope of practice at every stage.

Use it as your working reference for every visit — but bring your humanity, your intuition, and your training into the room with you. When in doubt, ask your supervisor.

How Clients Enter Postpartum Services

There are two pathways a client may come to you for postpartum doula support. Understanding which pathway your client is on will determine your starting point.

Pathway 1 — Postpartum-Only Clients

These clients were not matched with a birth doula through MHN. They are entering our services specifically for postpartum support. Your visit sequence is:

1. Virtual introductory visit
2. 72-hour postpartum visit (in-hospital or virtual)
3. 2 standard postpartum visits (in-home, before 8 weeks)
4. Up to 4 extended postpartum visits if requested (in-home, 8 weeks through 3 months)

Pathway 2 — Birth Clients Transitioning to Extended Postpartum

These clients were supported by an MHN birth doula during pregnancy and delivery. Their birth doula handled the 72-hour visit and standard postpartum visits through 8 weeks. If the client requests extended support, a postpartum doula is assigned. Your visit sequence is:

1. Virtual introductory visit
2. Up to 4 extended postpartum visits (in-home, 8 weeks through 3 months)

Doula Tip

You are meeting this client for the first time. You were not present for their birth or early postpartum period. This means the virtual intro visit and your first in-home visit are especially important for building trust and getting up to speed on their history.

Birth Doula Handoff Process (Pathway 2 Only)

Before your virtual introductory visit with a transitioning birth client, MHN admin staff will facilitate a handoff based on what is available in the client record. Not all handoffs will include every detail listed below — work with what you receive and plan to fill in any gaps through your own conversations with the client.

INFORMATION THE HANDOFF MAY INCLUDE

- **Birth summary** — type of delivery, any complications, how the client felt about their birth experience
- Feeding method and any challenges encountered (latch issues, low supply, supplementation, pumping)
- Emotional status throughout the postpartum period — any concerns flagged, mood changes observed, suggestions already given
- Family dynamics — partner involvement, other children, family support level, any tensions or stressors noted
- Cultural or religious considerations relevant to care
- Any ongoing concerns or things to watch for

- What went well — what the client responded to, what kind of support they seemed to value most

Doula Tip — Building Trust After a Handoff

Even with a thorough handoff, do not assume you know this client. The handoff gives you context, but your relationship with them starts fresh. Let them tell you things in their own words even if you already know. They need to feel heard by you, not just “briefed on.” If the handoff was limited, that's okay — your virtual intro visit and first in-home visit are where you'll build your understanding of their needs.

Visit 1: Virtual Introductory Visit (All Clients)

Format: Virtual (phone or video) **Timing:** During pregnancy (Pathway 1) or upon assignment after 8 weeks (Pathway 2) **Purpose:** Required before any in-person services, for safety and to build trust

What to Do During This Visit

Introduce yourself. Share your name, your role as a postpartum doula, and a little about your background. Keep it warm and human. This is their first impression of you.

Let the client share. For Pathway 1 clients, ask about their pregnancy, their birth plan or birth experience if baby has arrived, their concerns, and what kind of support they're hoping for. For Pathway 2 clients, acknowledge that you've reviewed their record, but invite them to share in their own words how things have been going. Ask what they feel they need most right now.

Discuss practical details for in-home visits. Pets in the home, shoes on or off, other children or family members who will be present, any cultural or religious practices you should be aware of, and their comfort level with various types of hands-on support.

Review the visit schedule. For Pathway 1, walk through the full sequence (72-hour visit, 2 standard visits, and the option for extended support). For Pathway 2, explain that they'll have up to 4 visits between now and 3 months postpartum and discuss how they'd like to space them.

Confirm logistics. Contact information, preferred communication method, their address, best times for visits, and how they'd like you to handle arrival (knock, ring, text when you're outside).

Ask about feeding. What method they're using, how it's going, any challenges. This helps you prepare for what support to bring.

Ask about their emotional state. How they're sleeping, how they're feeling overall, whether they have people they can talk to. For Pathway 2 clients especially, a lot may have shifted between 6 and 8 weeks.

Ask about mental health history they're comfortable sharing. Prior depression or anxiety, history of perinatal mood disorders, current medications or therapy. You are not their therapist, but this context helps you know what to watch for.

Pathway 1 Clients — Confirm the Follow-Up Appointment

Confirm that the client has their postpartum follow-up appointment scheduled with their doctor. If they do not have one scheduled, encourage them to get it on the calendar as soon as possible. This appointment is important for physical recovery and for screening for postpartum mood disorders.

Document the visit in the client record.

Visit 2: 72-Hour Postpartum Visit (Pathway 1 Only)

Format: In-hospital or virtual (depending on client circumstances and preference)

Timing: Within 72 hours of delivery **Purpose:** Early assessment, immediate support, and connection

What to Do During This Visit

Check in on physical recovery. Ask how they're feeling physically. Are they managing pain? Are they mobile? Have they been able to eat and drink? If they had a cesarean, are they able to get in and out of bed? You are not assessing them medically — you are listening and normalizing.

Assess emotional state. Are they tearful, overwhelmed, anxious, euphoric, numb, disconnected, or some combination? All of these can be normal in the first 72 hours. Note what you observe without labeling it. If something concerns you, document it and keep watching.

Support feeding. If breastfeeding, assist with positioning and latch. Watch a full feed if they're comfortable. Note any early difficulties — pain with latch, baby not staying awake, flat or inverted nipples, engorgement beginning. If challenges are significant, suggest they request the hospital's lactation consultant before discharge. If bottle feeding, support without judgment — help with paced bottle feeding technique and formula preparation if needed. If pumping, help them get started and understand colostrum collection.

If you are in the hospital: Help the client navigate the environment. Hospitals can be overwhelming, especially for first-time parents. Help them feel empowered to ask their nurses questions, to request help, to advocate for themselves. Be a calm presence in a clinical space.

Normalize the postpartum experience. Talk about what the next few days and weeks will look like — baby blues (typically days 3–5), cluster feeding, sleep disruption, physical soreness, hormonal shifts, the emotional rollercoaster of early parenthood. Reassure them that what they're feeling is common while also giving them permission to say if something doesn't feel right.

Include family members. If their partner or family is present, bring them into the conversation. Explain how they can help — taking night shifts, managing visitors, making sure the client eats, watching for warning signs. Help set realistic expectations now.

Provide resource information. Make sure the client is aware of WIC (if applicable), the pediatrician follow-up schedule, and how to reach you between visits. If they haven't yet scheduled their postpartum follow-up appointment with their doctor, remind them again.

Preview what comes next. Let them know when your next visit will be and what it will look like.

If This Visit Is Virtual

Focus on the emotional check-in and feeding assessment (ask them to show you a feed if comfortable). Ask about their home setup. Ask who is with them and what support they have in these first days. Plan for what they might need when you arrive for the first in-home visit.

Document the visit.

Visits 3 & 4: Standard Postpartum Visits (Pathway 1 Only)

Format: In-home, up to 2 hours each **Timing:** Between the 72-hour visit and 8 weeks postpartum **Purpose:** Core early postpartum support — confidence building, education, and observation

The client is in the thick of the adjustment — sleep deprived, possibly struggling with feeding, figuring out a new identity, and physically recovering. You are there to help them build confidence, feel less alone, and catch anything that needs attention.

Emotional Support

Listen actively. Let them talk, vent, cry, laugh, or just sit quietly with you. Don't rush to fix. Sometimes the most valuable thing you do in a visit is hold space.

Screen informally for perinatal mood and anxiety disorders. You are not diagnosing. You are observing. Watch for:

- Excessive crying beyond the first two weeks
- Inability to sleep even when the baby is sleeping
- Intrusive or scary thoughts
- Feelings of rage or irritability that feel out of proportion
- Panic attacks
- Detachment from the baby
- Feelings of worthlessness or guilt
- Loss of interest in things they normally enjoy
- Hypervigilance or inability to let anyone else hold or care for the baby

Perinatal Mood and Anxiety Disorders (PMADs)

An umbrella term for depression, anxiety, and related mood conditions that can occur during pregnancy or in the postpartum period, ranging from mild to severe.

Doula Script — Validating Their Experience

Avoid minimizing statements like “at least the baby is healthy” or “it gets easier.” Instead, try:

"This is really hard, and you're doing an incredible job showing up for your baby every day."

Encourage honesty. Ask how they're really doing, not just how the baby is doing. Many clients default to talking about the baby because they think that's what matters. Remind them that they matter too.

Feeding Support

Breastfeeding: Assist with positioning and latch. Help troubleshoot common challenges — sore or cracked nipples, engorgement, slow or forceful letdown, oversupply or low supply concerns, baby falling asleep at the breast, cluster feeding exhaustion. Teach hand expression if helpful. Support the client in understanding feeding cues and frequency.

Bottle feeding: Support without judgment regardless of reason. Help with paced bottle feeding technique to prevent overfeeding and gas. Assist with formula preparation and storage. Discuss responsive feeding — watching baby's cues rather than the clock.

Pumping: If the client is exclusively pumping or supplementing with pumped milk, help them establish and maintain a pumping schedule. Review milk storage guidelines. Help troubleshoot pump fit and output concerns.

Combination feeding: Many clients use a mix. Help them find a rhythm that works and manage the logistics without guilt.

If feeding challenges are beyond your scope, suggest the client connect with an IBCLC or lactation counselor.

IBCLC

International Board Certified Lactation Consultant — a clinically trained lactation specialist qualified to manage complex breastfeeding challenges beyond a doula's scope.

Newborn Care Education

Demonstrate and assist with hands-on care — diapering, swaddling, bathing (sponge and tub), soothing techniques (the 5 S's, skin-to-skin, wearing baby).

Discuss and reinforce safe sleep practices — baby on their back, on a firm flat surface, in their own sleep space, no loose blankets, pillows, bumpers, or stuffed animals.

Help the client learn to read their baby's cues — hunger (rooting, hand to mouth, fussing before crying), tiredness (yawning, looking away, fussiness), and overstimulation (arching, turning away, crying that escalates quickly).

Discuss normal newborn behavior so they know what to expect — cluster feeding, the witching hour, growth spurts and their timing, normal sleep-wake patterns, normal stool patterns for breast and formula fed babies, and common things that look alarming but are normal (baby acne, cradle cap, startle reflex, irregular breathing during sleep).

Assist with umbilical cord care and basic newborn hygiene.

Evidence-Based Information Sharing

As a doula, you may share evidence-based information from reputable, peer-reviewed sources on topics such as sleep practices, feeding methods, and newborn care. This is informational support, not prescriptive advice. You do not tell clients what they should or should not do. You provide them with the best available information so they can make informed decisions for their family.

Guiding Principle

If a client is already engaging in a practice — such as co-sleeping — your role is not to tell them to stop. Your role is to share current, evidence-based guidance from reputable sources (such as the AAP, La Leche League, or peer-reviewed journals) on how to do it as safely as possible. Harm reduction through education is always preferable to judgment.

Do not offer unsolicited opinions on parenting choices including co-sleeping, circumcision, vaccination schedules, sleep training methods, or feeding decisions. If a client asks for your input or if you observe a practice where safety information would be beneficial, share the evidence and let them decide.

Physical Recovery Support

Encourage rest. Help them strategize ways to sleep or rest when possible. If a partner or family member is present, help redistribute tasks so the client can nap.

Nourishment support — prepare a snack or drink, warm up a meal, make sure they're eating and hydrating. Many postpartum clients forget to eat or don't have free hands. Having something ready for them when you arrive or during the visit is a small thing that makes a big difference.

Encourage gentle movement as appropriate but do not push exercise. Recovery timelines vary, especially for cesarean births.

If the client has not yet attended their postpartum follow-up appointment with their doctor, check in about this and encourage them to keep or schedule that appointment.

Light Household Support

You may do light tidying in the areas where you're working — wash bottles, load or unload the dishwasher, fold a basket of laundry, straighten the nursery or living room, take out trash if it's overflowing.

This is not deep cleaning or housekeeping. Your primary role is supporting the client and baby. Light household tasks should only be done if they directly help the client rest, feed the baby, or feel less overwhelmed by their immediate environment.

Sibling Support

If older children are present, engage with them as needed so the client can focus on the baby or rest. Read a book, do a puzzle, involve them in gentle baby-related activities. Help normalize the transition and validate that big feelings about a new sibling are okay.

This is supplemental support, not babysitting. Your primary client is the postpartum parent.

Partner and Family Education

Include partners and family members in conversations when appropriate. Teach them soothing techniques, how to support feeding, safe sleep practices, and how to recognize warning signs in the client's mood or behavior. Encourage shared responsibility. Help set realistic expectations — the first weeks are about survival, not perfection.

What You Should NOT Do During Any Visit

- Do not provide medical advice or diagnose any condition
- Do not administer medication
- Do not perform clinical assessments (vitals, wound checks, etc.)
- Do not make calls or place referrals on behalf of the client — you may give recommendations and suggestions, but the client takes action themselves
- Do not offer unsolicited opinions on parenting choices — support their informed decisions with evidence-based information when asked or when safety is a concern
- Do not stay beyond the scheduled visit time without prior approval from your MHN supervisor
- Do not share the client's personal information with anyone outside of the MHN care team

Document every visit in the client record.

Extended Postpartum Visits (Both Pathways — 8 Weeks Through 3 Months)

Format: In-home, up to 2 hours each **Visits available:** Up to 4 **Timing:** After the 8-week mark through 3 months postpartum **Eligibility:** Client must request extended support; provided based on doula availability and location

Why Extended Support Matters

A lot happens between 8 weeks and 3 months. The initial wave of family help has often dried up. Partners may be back at work. The novelty of a new baby has faded for the outside world, but the client is still in the trenches. This period brings its own set of challenges that are different from the early postpartum days, and in many ways, this is when clients feel the most invisible.

Common challenges during this window include:

- Return to work and the emotional and logistical upheaval that comes with it
- Childcare transitions and separation anxiety
- Changes in milk supply (especially around 3 months when supply regulates and letdown may change)
- Delayed-onset postpartum mood disorders — postpartum depression and anxiety can emerge or intensify well after 6 weeks
- Relationship strain as the reality of new parenthood settles in
- Increased isolation as visitors stop coming and routines become repetitive
- Identity shifts and grief over the pre-baby self
- Physical recovery setbacks or pelvic floor issues that are only now becoming apparent

Your role during extended visits is to continue providing emotional, informational, and practical support while also watching for things that may need attention — because this is the window where problems that were quietly building can become crises.

Emotional Well-Being — This Is Your Top Priority

Reassess the client's emotional state at every visit. Do not assume that because they seemed fine at 6 weeks, they are fine at 10 weeks. Postpartum mood disorders can emerge late or worsen over time. Be especially watchful for:

- Persistent sadness, hopelessness, or emptiness that isn't lifting
- Anxiety that interferes with daily functioning — unable to leave the house, constant worry about the baby's health, checking on the baby excessively, catastrophic thinking
- Rage or irritability that feels out of control or out of character
- Intrusive thoughts — unwanted, disturbing thoughts about harm coming to the baby or themselves. These are more common than clients realize and are often a symptom of postpartum OCD, not a sign of danger, but they need professional support
- Difficulty bonding with the baby — feeling disconnected, going through the motions, not feeling the love they expected
- Withdrawal from partner, family, friends
- Loss of appetite, inability to sleep even when exhausted, or sleeping excessively
- Panic attacks
- Feelings of being a bad parent, guilt, shame

Any mention or hint of self-harm or suicidal ideation — this requires immediate action (see Scope of Practice section).

Ask direct but gentle questions:

Doula Script — Opening the Door

"How are you really feeling?"

"Are you getting any time for yourself?"

"Is there anything scaring you or keeping you up at night that isn't the baby?"

"Do you feel like yourself?"

For Pathway 2 clients especially, you are building this relationship from scratch. The client may not trust you enough to be vulnerable in the first extended visit. That's okay. Be consistent, be warm, and keep asking. Trust takes time.

Feeding Support

By 8 weeks to 3 months, feeding is often more established but new challenges emerge:

- Supply regulation around 3 months — breasts may feel softer and the client may worry their supply dropped. Educate them that this is normal and supply is likely fine if baby is gaining weight and producing adequate diapers
- Pumping at work — if returning to work, help them plan a pumping schedule, understand their rights to pump breaks and a private space, troubleshoot pump output concerns, and manage milk storage and transport
- Bottle refusal or breast refusal — babies sometimes develop preferences. Troubleshoot with patience
- Introduction of solids readiness — around 4–6 months (closer to the end of your service window). Discuss signs of readiness but do not push early introduction
- Weaning — if the client is choosing to wean, support them emotionally and practically. Weaning can trigger hormonal shifts and grief. Do not judge their decision
- Formula transitions — if switching formulas, help them understand what to expect

If feeding challenges are beyond your scope, suggest the client connect with an IBCLC or lactation counselor.

Return to Work Transition

For clients returning to work during this window, the transition is often one of the hardest things they've faced. Support might include:

- Emotional processing — guilt, sadness, relief, anxiety, all of it is normal and often all of it happens at once
- Practical planning — pumping schedule, what to pack, how to handle daycare drop-off, how to maintain milk supply
- Childcare adjustment — helping them prepare for separation, what to expect in the first weeks, how to evaluate if the arrangement is working
- Evening routine planning — how to reconnect with baby after work, manage household tasks, and still get rest

- Knowing their rights — help them understand their rights around pumping breaks and lactation accommodations at work

Infant Development and Changing Needs

Between 8 weeks and 3 months, babies change rapidly. Help the client understand and adapt to:

- Emerging social smiles and increased alertness — baby is more interactive, which is rewarding but also more demanding
- Changing sleep patterns — the 3–4 month sleep regression may begin near the end of this window. Help them understand what's happening developmentally and set realistic expectations
- Increased fussiness or colic that may peak around 6–8 weeks and begin to improve by 3 months
- Tummy time, early motor development, and appropriate play and stimulation
- Growth spurts and their effect on feeding and sleep
- When to be concerned vs. when to trust the process

Evidence-Based Information Sharing

The same principles from standard visits apply during extended support. You may share evidence-based, peer-reviewed information on topics such as sleep methods, infant development milestones, feeding transitions, and safe practices. Your role is to inform, not instruct. If a client is engaging in a practice, provide the best available evidence so they can do so as safely and confidently as possible.

As babies grow and new questions arise — about sleep training approaches, introducing solids, screen time, and more — the client may look to you for guidance. Share what the research says from reputable sources. Let them make the decision.

Relationship and Family Dynamics

The 8 week to 3 month period often puts strain on partnerships and family relationships. You may observe or hear about:

- Conflict between partners about division of labor
- Differing parenting philosophies between the client and their partner or extended family
- Resentment building on one or both sides

- Intimacy concerns (physical and emotional)
- Unsolicited advice from family members that is causing stress
- Isolation from friends or community

You are not a couples' therapist. But you can validate what the client is experiencing, normalize the strain, encourage honest communication, and suggest they explore counseling resources if they feel it would help.

Light Household Support

Light household support remains an important part of extended visits. A cluttered or chaotic home environment is directly tied to emotional well-being during the postpartum period. When a client is already overwhelmed, a sink full of bottles or laundry piling up can compound feelings of failure, anxiety, and being out of control.

During extended visits, continue to offer light household help as part of your holistic support:

- Wash and sterilize bottles
- Load or unload the dishwasher
- Fold a load of laundry
- Tidy the nursery, kitchen, or living room
- Take out trash or recycling
- Prepare a snack or drink, warm up a meal

This is still not deep cleaning or housekeeping. But during the extended period, when outside help has often disappeared entirely, these small acts of practical care can be profoundly meaningful. Don't underestimate their impact on a client's mental state.

Building Sustainable Routines

Help the client begin to build routines that will carry them beyond your services:

- Sleep strategies that work for their family
- Self-care habits that are realistic, not aspirational — even 10 minutes of something restorative counts
- Learning to ask for and accept help
- Connecting with community — local parent groups, MHN support resources, faith communities, online groups for their specific situation
- Establishing their own rhythm rather than comparing themselves to others

Preparing for Closure of Services

As you approach the final visit, begin transitioning:

- Share information about ongoing resources available to them — community programs, support groups, hotlines, their pediatrician and their own doctor as continued touchpoints
- Affirm their growth and capability. Be specific about what you've observed — name the ways they've shown up for their baby and themselves
- Suggest any final connections they might benefit from — mental health support, lactation support, pelvic floor physical therapy, community resources
- Let them know it's okay to still have hard days after your visits end. Having a doula end services doesn't mean they should have it all figured out

Documentation Requirements

After every visit, complete your documentation in the MHN client record. At minimum, record:

Documentation Checklist

- Date, time, and duration of the visit
- Visit format (virtual, in-hospital, in-home)
- Brief summary of what was discussed, observed, and done
- Feeding status and any changes or concerns
- Emotional and mental health observations — note specific behaviors or statements, not just “seemed fine”
- Any concerns observed (emotional, feeding, safety, family dynamics)
- Recommendations or suggestions given to the client and whether they indicated they would follow up
- Plan for the next visit or, if final visit, closure summary

Good documentation protects the client, protects you, and ensures continuity if another doula ever needs to step in.

Scope of Practice

You are a non-clinical support person. Your value is enormous — research consistently shows that doula support improves outcomes for birthing people and babies. But your lane is emotional, informational, and practical support. You give recommendations and suggestions. You share evidence-based information from reputable sources. You do not make calls, place referrals, or take clinical action on behalf of the client. The client is the decision-maker, and you are their informed, compassionate guide.

When in doubt, flag it to your supervisor. When something concerns you, say something.

When to Suggest the Client Contact Their Medical Provider

If you observe any of the following, recommend that the client reach out to their OB, midwife, or go to the ER:

Medical Warning Signs

- Fever or chills
- Excessive or worsening bleeding
- Foul-smelling vaginal discharge
- Signs of infection at a cesarean incision or perineal tear (redness, swelling, warmth, oozing)
- Severe headache or vision changes
- Chest pain or difficulty breathing
- Calf pain or swelling (possible blood clot)
- Difficulty urinating or signs of urinary tract infection
- Any acute symptom that feels emergent

Be clear and direct in your recommendation:

Doula Script — Medical Referral

"This is something I'd really encourage you to call your doctor about today."

"I think this warrants a call to your provider — would you feel comfortable doing that?"

When to Suggest the Client Connect with a Mental Health Professional

If you observe any of the following, suggest the client explore connecting with a perinatal mental health professional:

Mental Health Warning Signs

- Persistent sadness or depressed mood lasting more than 2 weeks
- Anxiety that is interfering with their ability to function or care for themselves or baby
- Intrusive thoughts about harm to themselves or the baby
- Inability to sleep even when the baby is sleeping and they have the opportunity
- Complete loss of interest in the baby or in activities they previously enjoyed
- Feelings of hopelessness, worthlessness, or excessive guilt
- Panic attacks
- Detachment from reality or feeling like things are not real
- Severe mood swings that are not improving
- Any behavior that feels "off" even if you cannot name exactly what is wrong — trust your gut

Doula Script — Mental Health Referral

"What you're describing sounds really overwhelming, and I think it would help to talk to someone who specializes in postpartum mental health. Would you be open to looking into that?"

Crisis Situations

If the Client Expresses Suicidal Thoughts or Intent to Harm Themselves or Their Baby

This is a crisis.

Stay calm. Do not leave them alone if possible. Encourage them to contact the 988 Suicide and Crisis Lifeline (call or text 988), their provider, or emergency services. Notify your MHN supervisor immediately. Document everything.

When to Contact Your MHN Supervisor

Reach out to your supervisor whenever:

- You observe something concerning and are unsure how to proceed
- You have safety concerns about the home environment
- A client misses multiple scheduled visits or becomes unreachable
- You are unsure whether something falls within your scope
- A client's needs exceed what you can provide in the allotted visits
- You need to coordinate a handoff or service extension

Doula Tip

You are never expected to handle a crisis alone. Your supervisor is your resource — use them.

Final Notes

Key Takeaways

- Every family is different, and every visit will look different — some visits are spent entirely on breastfeeding support, some holding the baby so the client can shower, some just sitting with them while they cry. All of those visits matter
- You are often the first person outside of the medical system to really see this client and ask how they are doing — not how the baby is doing, how they are doing
- That simple act of seeing them, asking, and listening without judgment is more powerful than you may realize
- Use this guide as your framework, but bring your humanity, your intuition, and your training into every visit
- Trust what you've learned. Trust what you observe. And when in doubt, ask for help